

1.3.1	Desloratadine
SPC, Labeling and Package Leaflet	MY

Dasselta

Desloratadine

NAME OF THE MEDICINAL PRODUCT

Dasselta 5 mg film-coated tablets

COMPOSITION

Each tablet contains 5 mg desloratadine.

PRODUCT DESCRIPTION

Dasselta film-coated tablets are light blue, round with beveled edges.

THERAPEUTIC INDICATIONS

Dasselta tablets are indicated for the rapid relief of symptoms associated with allergic rhinitis, such as sneezing, nasal discharge, itching, congestion/stuffiness, as well as ocular itching, tearing and redness.

Dasselta tablets are also indicated for the relief of symptoms associated with chronic idiopathic urticaria, such as the relief of itching and the size and number of hives.

POSODOLOGY AND METHOD OF ADMINISTRATION

Adults and adolescents (12 years of age and over): One Dasselta 5 mg film-coated tablet once a day regardless of mealtime for the relief of symptoms associated with allergic rhinitis (including intermittent and persistent allergic rhinitis) and urticaria.

For oral use.

Intermittent allergic rhinitis (presence of symptoms for less than 4 days per week or for less than 4 weeks) should be managed in accordance with the evaluation of patient's disease history and the treatment could be discontinued after symptoms are resolved and reinitiated upon their reappearance. In persistent allergic rhinitis (presence of symptoms for 4 days or more per week and for more than 4 weeks), continued treatment may be proposed to the patients during the allergen exposure periods.

CONTRAINDICATIONS

Hypersensitivity to the active substance, to any of the excipients or to loratadine.

SPECIAL WARNINGS AND PRECAUTIONS FOR USE

In the case of severe renal insufficiency, Dasselta should be used with caution.

Dasselta contains lactose. Patients with rare hereditary problems of galactose intolerance, the Lapp lactase deficiency or glucose-galactose malabsorption should not take this medicine.

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INTERACTION WITH OTHER MEDICINAL PRODUCTS AND OTHER FORMS OF INTERACTION

No clinically relevant interactions were observed when desloratadine tablets were co-administered with erythromycin or ketoconazole.

Desloratadine taken concomitantly with alcohol did not potentiate the performance impairing effects of alcohol. However, caution is recommended if alcohol is taken concomitantly.

FERTILITY, PREGNANCY AND LACTATION

Pregnancy

There are no or limited amount of data from the use of desloratadine in pregnant women. Animal studies do not indicate direct or indirect harmful effects with respect to reproductive toxicity. As a precautionary measure, it is preferable to avoid the use during pregnancy.

Breast-feeding

Desloratadine has been identified in breastfed newborns/infants of treated women. The effect of desloratadine on newborns/infants is unknown. A decision must be made whether to discontinue breast-feeding or to discontinue/abstain from desloratadine therapy taking into account the benefit of breast feeding for the child and the benefit of therapy for the woman.

Fertility

There are no data available on male and female fertility.

EFFECTS ON ABILITY TO DRIVE AND USE MACHINES

Desloratadine has no or negligible influence on the ability to drive and use machines. Patients should be informed that most people do not experience drowsiness. Nevertheless, as there is individual variation in response to all medicinal products, it is recommended that patients are advised not to engage in activities requiring mental alertness, such as driving a car or using machines, until they have established their own response to the medicinal product.

UNDESIRABLE EFFECTS

Tabulated list of adverse reactions

System Organ Class	Frequency	Adverse reactions seen with desloratadine
Psychiatric disorders	Very rare	Hallucinations
Nervous system disorders	Common Very rare	Headache Dizziness, somnolence, insomnia, psychomotor hyperactivity, seizures
Cardiac disorders	Very rare Not known	Tachycardia, palpitations QT prolongation
Gastrointestinal disorders	Common Very rare	Dry mouth Abdominal pain, nausea, vomiting, dyspepsia, diarrhoea
Hepatobiliary disorders	Very rare Not known	Elevations of liver enzymes, increased bilirubin, hepatitis Jaundice

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Skin and subcutaneous skin disorders	Not known	Photosensitivity
Musculoskeletal and connective tissue disorders	Very rare	Myalgia
General disorders and administration site conditions	Common Very rare	Fatigue Hypersensitivity reactions (such as anaphylaxis, angioedema, dyspnoea, pruritus, rash, and urticaria)

OVERDOSE

In the event of overdose, consider standard measures to remove unabsorbed active substance. Symptomatic and supportive treatment is recommended.

Doses up to 45 mg of desloratadine (nine times the clinical dose), had no clinically relevant effects, including no prolongation of QTc interval.

Desloratadine is not eliminated by haemodialysis; it is not known if it is eliminated by peritoneal dialysis.

PHARMACODYNAMIC PROPERTIES

Pharmacotherapeutic group: Other antihistamines for systemic use, ATC code: R06AX27

Mechanism of action

Desloratadine is a non-sedating, long-acting histamine antagonist with selective peripheral H₁-receptor antagonist activity. After oral administration, desloratadine selectively blocks peripheral histamine H₁-receptors because the substance is excluded from entry to the central nervous system.

Desloratadine has demonstrated antiallergic properties from in vitro studies. These include inhibiting the release of proinflammatory cytokines such as IL-4, IL-6, IL-8, and IL-13 from human mast cells/basophils, as well as inhibition of the expression of the adhesion molecule P-selectin on endothelial cells. The clinical relevance of these observations remains to be confirmed.

PHARMACOKINETIC PROPERTIES

Absorption

Desloratadine plasma concentrations can be detected within 30 minutes of administration. Desloratadine is well absorbed with maximum concentration achieved after approximately 3 hours; the terminal phase half-life is approximately 27 hours. The degree of accumulation of desloratadine was consistent with its half-life (approximately 27 hours) and a once daily dosing frequency. The bioavailability of desloratadine was dose proportional over the range of 5 mg to 20 mg.

In a pharmacokinetic trial in which patient demographics were comparable to those of the general seasonal allergic rhinitis population, 4 % of the subjects achieved a higher concentration of desloratadine. This percentage may vary according to ethnic background. Maximum desloratadine concentration was about 3-fold higher at approximately 7 hours with a terminal phase half-life of approximately 89 hours. The safety profile of these subjects was not different from that of the general population.

Distribution

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Desloratadine is moderately bound (83 % - 87 %) to plasma proteins. There is no evidence of clinically relevant medicine accumulation following once daily dosing of desloratadine (5 mg to 20 mg) for 14 days.

Biotransformation

The enzyme responsible for the metabolism of desloratadine has not been identified yet, and therefore, some interactions with other medicinal products can not be fully excluded. Desloratadine does not inhibit CYP3A4 *in vivo*, and *in vitro* studies have shown that the medicinal product does not inhibit CYP2D6 and is neither a substrate nor an inhibitor of P-glycoprotein.

Elimination

In a single dose trial using a 7.5 mg dose of desloratadine, there was no effect of food (high-fat, high caloric breakfast) on the disposition of desloratadine. In another study, grapefruit juice had no effect on the disposition of desloratadine.

LIST OF EXCIPIENTS

The ingredients in the tablet core are: microcrystalline cellulose, hypromellose, hydrochloric acid, sodium hydroxide, maize starch, lactose monohydrate and talc.

The ingredients in the film-coating are: hypromellose, macrogol, lactose monohydrate, titanium dioxide (E171) and indigo carmine (E132).

SPECIAL PRECAUTIONS FOR STORAGE

Do not store above 30 °C.

Store in the original package in order to protect from moisture.

PRESENTATION

Blisters packs: 10 or 100 tablets in a folding box.

Not all pack sizes may be marketed.

SPECIAL PRECAUTIONS FOR DISPOSAL

Any unused medicinal product or waste material should be disposed of in accordance with local requirements.

PRODUCT REGISTRATION HOLDER

PAHANG PHARMACY SDN. BHD.

Lot 5979, Jalan Teratai, 5 ½ Miles, Off Jalan Meru, 41050 Klang, Selangor, Malaysia

NAME AND ADDRESS OF THE MANUFACTURER

KRKA, d.d., Novo mesto, Šmarjeska cesta 6, 8501 Novo mesto, Slovenia

DATE OF REVISION OF THE TEXT

August, 2020

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